# INCIDENT HANDLER'S JOURNAL

**ENTRY 01 · RANSOMWARE INCIDENT · HEALTHCARE CLINIC**

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| DATE 2026-05-22 | ENTRY 01 |
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| DESCRIPTION | Documentation of a ransomware incident affecting a small U.S. healthcare clinic specializing in primary care. Attackers gained initial access through targeted phishing emails containing a malicious attachment; once executed, ransomware encrypted critical files including patient medical records, halted business operations, and prompted a ransom demand in exchange for the decryption key. |
| TOOLS USED | None directly engaged at this documentation stage; this entry captures initial incident intake and triage notes.<br>**Anticipated response toolset:**<br>SIEM platform · EDR / antivirus · email security gateway logs · network traffic analyzer (Wireshark or Zeek) · forensic imaging utilities · backup integrity verification · incident handler's journal. |
| THE 5 W'S | **WHO** An organized threat group known to target organizations in the healthcare and transportation sectors.<br>**WHAT** Ransomware deployment following phishing-based initial access. Critical files were encrypted and a ransom note demanded payment for the decryption key.<br>**WHEN** Tuesday at approximately 9:00 a.m. local time, when employees attempted to begin the workday and discovered they could not access files.<br>**WHERE** A small primary-care clinic in the United States. Impact concentrated on employee workstations and the file shares holding medical records and operational software.<br>**WHY** Targeted phishing with a malicious attachment exploited the human layer. Once the attachment was downloaded, malware established a foothold and the attackers escalated to ransomware deployment across |

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| | networked systems. |
| ADDITIONAL NOTES | Healthcare is a high-value target due to time-sensitive patient data and the operational pressure to restore access quickly. Both factors raise the likelihood that a victim will pay the ransom.<br><br>**Open questions for further investigation:**<br><br>· Was multi-factor authentication enforced on employee email accounts?<br>· Were backups segmented, offline, and routinely tested for restorability?<br>· Were endpoints running EDR with behavioral detection capable of catching ransomware execution patterns?<br>· Did the organization have a documented incident response plan and assigned roles before the event?<br><br>**Applicability to CTA's North Idaho client base:**<br><br>Phishing-aware training, segmented backups, and email attachment filtering are inexpensive controls that materially reduce blast radius for small organizations. These map directly to the kinds of recommendations Community Tech Aid is positioned to deliver to small healthcare practices, nonprofits, and similar operations in the region. |